

2006 CanLII 11911 (ON SC)

Rajesh Datt, for the Defendant

MECHANISM OF THE ACCIDENT

[2] I find Ms. Garratt to be a very credible witness who testified in a forthright manner, without exaggeration. She has a grade 10 education. She has been a motivated hard-worker all of her life, working mainly in the cleaning business. She was upset and distraught after this accident in which her car was “lassoed” by a cable (others have testified that it was a spider rope) which dropped down from the Highway #11 overpass in Orillia. This was a bizarre accident and it is understandable that Ms. Garratt would have some confusion over the details. There were no independent witnesses and the Orillia Power employees working at the site were away for lunch at the time of the incident.

[3] Ms. Garratt testified that, as she was driving under the overpass, she saw a cable come down two or three feet away from her. She braked “really quick, a couple of seconds”. After she applied her brakes, she never reapplied the accelerator. The cable hit the centre of her bumper, came up over the hood, over the windshield and when it got to her driver’s side rear view mirror it got wrapped around the mirror. She took her hands off the wheel and feet off the pedals, as she thought she was going to get electrocuted by what she thought was a live wire. She was terrified. She thought she was going to die. She travelled about 80 to 100 feet before she came to a sudden stop. She was thrown forward and back. She was wearing a seat belt.

[4] After she stopped she looked back and watched the cable loosen and release. Once the cable released she realized she had her foot on the brake. She pulled over to the shoulder. She looked in her rear view mirror and noticed a man winding up the cable, which was about one inch in diameter and grey coloured with strands of metal woven together.

[5] The Defendant disputes Ms. Garratt’s testimony about how the accident occurred and called Gordon Jenish, an engineering expert qualified in accident reconstruction and the forces occurring during an accident. The defendant’s position is that a spider rope and not a cable was involved with Ms. Garratt’s car. Mr. Jenish’s opinion was that the testing of the tensile strength of the spider rope indicated that the rope was incapable of exerting sufficient force on the Garratt vehicle to bring it to a stop alone. In cross-examination Mr. Jenish agreed that no testing was performed involving the combination of a rope causing resistance with braking or on resistance caused by a cable with braking. Further, he did not perform tests on the cable or the heavier gauged rope also used by the Defendant in one stage of its operations.

[6] I find that the object, which was released by the unknown man, was either/or a combination of the cable, the spider rope or the heavier gauged rope. No one will ever know precisely what happened, as there were no independent witnesses. The Defendants’ representatives were not in attendance at the time of the incident

[7] Ms. Garratt testified about the cable and the application of the brakes. In cross-examination, she said that the car stopped because of the cable wrapping around the mirror. Based on her evidence, defence counsel urges me to find that this was the sole ostensible reason for the stopping - not the combination of the cable/rope with braking. I do not make this finding. Ms. Garratt is not an engineer – she testified about the braking and I accept her evidence as to what happened.

THE POTENTIAL FOR THIS ACCIDENT TO HAVE CAUSED INJURIES

[8] Mr. Jenish testified that injuries are dependent upon the forces to which a vehicle was subjected during an accident and the resulting biomechanical forces acting on the occupants. He performed a number of tests at speeds in the range of 85 to 95 kilometers per hour using a car similar to that driven by Ms. Garratt. His tests measured the forces of gravity experienced during deceleration, during braking and during lateral acceleration (which one experiences with a car being steered in one direction or another, including abrupt lane changes). Mr. Jenish concluded that the maximum force experienced by a driver using a vehicle similar to that being driven by Ms. Garratt during a panic-braking stop was .8 G (G being one unit of measurement of the force of gravity).

[9] Mr. Jenish provided a number of everyday movements measured for their G force and opined that the forces experienced by Ms. Garratt during the accident were below a level that could be experienced during some common everyday experiences, such as being jostled in a crowd and coughing, which generated G levels of 1.8 G and 3.0 G respectively.

[10] The Defendant submits that the forces experienced by Ms. Garratt in the accident cannot explain her injuries. In cross-examination, Mr. Jenish agreed that the forces on a body within a car may not be exactly the same as the forces on the car itself. Scientific studies have shown that the acceleration of the neck and skull can be three to four times greater than the force on the car itself because of the action of the shoulders and head whiplashing. Ms. Garratt testified that her body was thrown forward and backward during this incident. There is no evidence of another accident or incident following this accident that can explain her injuries.

[11] Dr. Peter Parker, a medical doctor qualified in the area of physiatry, opined that no matter how the car was stopped, an acceleration and deceleration were involved, resulting in forces, injuries and impairments. He also opined that the effects of the accident have nothing to do with the forces on the car.

[12] Dr. Morris Charendoff, a medical doctor qualified in the area of orthopaedic surgery, also disagrees with Mr. Jenish's opinion that injuries are dependent upon the forces to which a vehicle was subjected during the collision. Dr. Charendoff related his experience with accident victims and opined that, in many accidents, there is no relationship between the extent of trauma to the vehicle and trauma to the occupants. The examples he provided included minor rear end collisions wherein occupants sustain extensive musculoskeletal trauma leading to chronic pain, and more extensive accidents in which the occupants sustain no significant injuries or only minor injuries.

[13] Dr. Gordon Hunter, a medical doctor qualified in the area of orthopaedic surgery, called by the defence, testified that "well, if someone drives along the road and has either a whiplash, or

a rear end, or stops the car suddenly to avoid hitting a deer, they suffer a flexion extension injury of their neck and their back to a lesser extent...so that's what she had, whatever – however she slowed, however fast she slowed or...”

THE RELATIONSHIP BETWEEN MS. GARRATT'S INJURIES AND THE MOTOR VEHICLE ACCIDENT

[14] Dr. Parker was of the opinion that Ms. Garratt's condition, restrictions and limitations were directly related to this accident.

[15] Dr. Charendoff believed that Ms. Garratt's chronic pain syndrome was triggered by the accident and the subsequent development of her symptoms in multiple areas of her body.

[16] Dr. Hunter opined, “I think she sustained soft tissue injuries of her neck, upper thoracic, and low back area as a result of the accident”.

[17] Dr. Tibor Harmathy, a medical doctor qualified in the area of family medicine and Ms. Garratt's family doctor of 22 years, provided the opinion that she sustained soft tissue injuries secondary to her accident.

[18] Mr. Jenish provided no opinion on the connection between the physical forces involved in the accident and Ms. Garratt's medical injuries. In fact, it was acknowledged by the defence that he was not qualified to offer an opinion on this issue.

LEGAL PRINCIPLES OF CAUSATION

[19] The usual test for causation is the “but for” test, which requires a Plaintiff to demonstrate that “but for” the negligence of the Defendant, he or she would not have suffered the injury or the loss. The Plaintiff is not required to establish that the Defendant's negligence was the sole cause of the injury. As long as the Defendant is part of the cause of an injury, the Defendant is liable, even though his act alone was not enough to create the injury. There is no basis for a reduction of liability because of the existence of other preconditions: Defendants remain liable for all injuries caused or contributed to by their negligence. (*Cork v Kirby MacLean Limited* [1952] 2 All E.R. 402 (C.A.) and *Athey v. Leonati* [1996] 3 S.C.R. 450.)

[20] Where the “but for” test is unworkable, causation can be established where the Defendant's negligence “materially contributed” to the injury. In order for a contribution to be material, it must exceed the de minimis range (*Athey v Leonati*).

[21] Where a Plaintiff proves on a balance of probabilities that the Defendant's negligent act materially contributed to the injury complained of, the Plaintiff satisfies the onus of proving causation (*Kersey v Wellesley Hospital* [1988] 46 C.C.L.T. 271 (Ont.H.C.J.)).

[22] The legal or ultimate burden remains with the Plaintiff, although positive or scientific proof of causation is not required. An inference of causation may be drawn by applying common sense to the evidence presented by the Plaintiff or in the absence of evidence to the contrary adduced by the Defendant. Even if the Defendant provides some evidence to the contrary, that evidence is to be weighed against the Plaintiff's evidence and causation can still be inferred (*Blatch v Archer* [1774] 98 E.R.969 as cited in *Snell v. Farrell* [1990] 2 S.C.R. 311).

[23] In the case of *Laferriere v Lawson* (1991) 78 D.L.R. (4th) 609 (S.C.C.) Gonthier J. for the Supreme Court of Canada confirmed the principles as set out in *Snell v Farrell*:

- (a) causation in law does not require scientific causation;
- (b) causation in law must be established on the balance of probabilities, taking into account all of the evidence: factual, statistical and that which the judge is entitled to presume;
- (c) in some cases, where a fault presents a clear danger and where such a danger materializes, it may be reasonable to presume a causal link, unless there is a demonstration or indication to the contrary; and
- (d) even where statistical and factual evidence do not support a finding of causation on the balance of probabilities with respect to particular damage, such evidence may justify a finding of causation with respect to lesser damage.

CONCLUSIONS ON CAUSATION

[24] I am satisfied that, based on all of the evidence, but for the negligence of Orillia Power, Ms. Garratt would not have suffered her injuries. The situation, created by Orillia Power, presented a clear danger (it was an insecure installation) and the danger materialized. Alternatively, the Plaintiff has established that the negligent act of Orillia Power materially contributed to Ms. Garratt's injuries. In making these findings, I am applying common sense to the evidence presented by the Plaintiff. I am persuaded by the opinions of Dr. Parker and Dr. Charendoff. Even the defence doctor, Dr. Hunter, provided evidence supporting the plaintiff's position on this accident and opined that it was of no matter how she slowed or how quickly she slowed.

[25] I accept the opinions of the medical experts. The timing and progression of the pain and injuries support a causal link to the accident: the accident is a material cause of Ms. Garratt's overall condition. Causation is proven and Ms. Garratt is entitled to full recovery of her damages, those damages being subject to other legal principles such as the assessment of pre-existing frailties dealt with below.

WHICH OF MS. GARRATT'S INJURIES RESULTED FROM THE ACCIDENT?

[26] In determining which of Ms. Garratt's injuries are related to the accident, it is necessary to start by describing them. She testified as to headaches occurring several times a week; daily neck pain particularly when turning her neck, which wakes her from sleep; continuous back pain, the intensity of which she described as 4 to 5 out of 10, worse at night and shooting when she moves; lack of energy; and depression. She has utilized physiotherapy, traction, ultrasound, massage, acupuncture, exercise and reiki therapy.

[27] Dr. Charendoff was of the opinion that Ms. Garratt sustained myofascial strains and sprains to the cervical spine (the neck), thoracic spine, lumbar spine, and right shoulder; bilateral soft tissue injuries to her knees; post-traumatic cervicogenic headaches; and chronic pain syndrome.

[28] Dr. Parker opined that she had limited cervical and lumbosacral spine range of movement; bilateral muscle spasms throughout the neck and upper back; and positive stress testing of the cervical facet, lower thoracic and lumbar spine.

[29] According to Dr. Hunter, Ms. Garratt sustained soft tissue injuries to her neck, upper thoracic and low back area.

[30] Dr. Harmathy opined that she sustained soft tissue injuries to her neck and back.

[31] Dr. Tad Crohn, a psychologist who was qualified as an expert in chronic pain, provided the opinion that Ms. Garratt developed a complicated chronic pain syndrome. As a result of the severe trauma and the unique situation in which she felt dismissed, lied to and invalidated by the Defendant through the process following the accident, she developed post-traumatic stress problems which went into a mutually reinforcing cycle with her soft tissue injuries. Dr. Crohn is the only doctor who was qualified as an expert in chronic pain. The defence psychiatrist, Dr. R.B. Hines, was not so qualified and he agreed at trial that he would defer to the opinion of an expert in chronic pain – of which Dr. Crohn is one. I accept that Ms. Garratt has chronic pain caused by the accident and that the problems set out above, with the exception of the bilateral soft tissue damage to her knees, are directly related to the accident.

[32] Ms. Garratt also testified as to constant hip pains, which she ranked at 3 or 4 out of 10, which arose in the early part of 2004. These pains affect her work, her dancing, and her gardening. She also testified as to intermittent knee pains, which she ranked at 4 out of 10, which arose in the summer of 2004. These pains also affect her work, dancing and gardening.

[33] Although Dr. Harmathy testified that pain patterns can change, he agreed that because of the timing it would be unusual for these problems to have developed as a result of the accident.

[34] Dr. Hunter does not believe that these problems are related to the accident.

[35] Ms. Garratt described burning in the front of her thighs and in her kneecaps to Dr. Parker. As there was no direct trauma to her knee joints, Dr. Parker felt that the burning in her lower extremities is related more to her work.

[36] Dr. Charendoff opined that her bilateral upper thigh, hip and knee pain are likely unattributable to the accident because of the timing of their development.

[37] There is no compelling evidence to establish a causal connection between the accident and these lower extremity problems.

DAMAGES

LYNDA GARRATT'S PRE-EXISTING MEDICAL CONDITION

[38] As noted by the Supreme Court in *Blackwater v. Plint* (2005) 258 D.L.R. (4th) 275, “[i]t is important to distinguish between causation as the source of loss and the rules of damage assessment in tort.” (at para. 78) The test for causation is as set out above. Once causation is established, “[t]he rules of damages then consider what the original position of the plaintiff would have been. The governing principle is that the defendant need not put the plaintiff in a better position than his original position and should not compensate the plaintiff for any damages he would have suffered anyway.” Damages may be reduced by reason of other causes contributing to Ms. Garratt’s overall condition if those causes would have manifested themselves in the absence of the tortious conduct.

[39] Much time was spent at trial on Ms. Garratt’s prior medical condition. Although there were some inconsistencies in her evidence regarding dates and the description of symptoms provided to the various physicians and other medical providers, Ms. Garratt was a forthright witness, doing her best to provide accurate information. It was apparent that she was seeing and reviewing some of her medical records for the first time at trial. Although the Defendant wants me to find that Ms. Garratt lied to her doctors and health practitioners about her pre-existing medical condition, I do not make that finding. A lot of time was spent on the questionnaire filled out by Ms. Garratt upon entering into treatment with Dr. Donald Munro, a chiropractor. The Defendant tried to establish that the various medical experts had not received this questionnaire, which counsel submits establishes that Ms. Garratt provided false information to the doctors and minimized her prior medical problems. This is patently untenable. The various doctors were sent Dr. Munro’s records which contained the questionnaire and they testified they reviewed the various records sent to them.

PRIOR BACK PROBLEMS

[40] In 1998 Ms. Garratt was working at Geneva Park as a cleaner/housekeeper. After bending over to scrub a mattress, her back locked as she went to stand up. She saw her family doctor who provided treatment. She testified that she was better after about two weeks.

[41] Dr. Harmathy confirmed that she was off work from October 14, 1998 until November 3, 1998 because of this injury.

[42] When Dr. Harmathy saw Ms. Garratt on October 20, 1998 she provided the details of the incident. He described the injury as “almost a mechanical dysfunction of the sacroiliac joint”, for which he utilized orthopaedic manipulation and acupuncture that day and also on October 22, 1998.

[43] On May 2, 2001, Dr. Harmathy performed a physical examination on Mrs. Garratt. There was no discussion about back problems. There had been no intervening appointment about her back between October of 1998 and May of 2001. On June 8, 2001, Dr. Harmathy noted some right shoulder soreness radiating to her neck and also down her arm. He noted arm and shoulder normal and range of motion normal. He treated her with manipulation. His notes that follow this visit do not note any continued complaints with her shoulder, although neck swelling caused by thyroid dysfunction is noted. Ms. Garratt’s thyroid problems are dealt with below.

[44] In July of 2001, Ms. Garratt attended with Dr. Lawrence LaFrance, a chiropractor, whose note of July 10, 2001, confirms a 3-week history of complaints regarding her neck, shoulder blade and right arm. She had some numbness and tingling in her arm. He questioned facet joint syndrome and thoracic outlook syndrome. His treatment involved electrical current manipulation and exercise. Dr. LaFrance provided treatment until July 27, 2001, with improvement noted. Eventually, she discharged herself, which indicated to him that she was better. An x-ray, taken on July 12, 2001, confirmed a finding that spasm from pain could be present but also confirmed no disc space narrowing or other abnormality.

[45] In August of 2002, while housecleaning, Ms. Garratt bent down to grab something and the back pain returned, running across her left side and down her leg. She sought treatment from Dr. Munro, who assessed her on August 6, 2002, and started treatment on August 7, 2002, which included a lumbosacral adjustment. Dr. Munro saw her many times. There is a note of September 9, 2002, which confirms that she had started an exercise program at Curves. The last visit was September 16, 2002, which resulted in a note that she had improved. He did not see her again. In the charted history, he noted “first bad episode about five or six years ago, since then re-occurred about one time a year not sure what caused last bad bout, approximately one year ago”. His chart notes chronic low back pain. Ms. Garratt testified that after discharging herself from Dr. Munro she joined Curves and became involved in an exercise program. By mid September 2002, she was back to full-time work and was fully functional. She had lost some weight and inches.

[46] Dr. Parker described her pre-accident level of function as extremely high. She ran her commercial cleaning business, cleaned for private clients and had a very active family and social life. The lay witnesses included family, friends and clients who all described Ms. Garratt as an active, hardworking go-go-go individual.

[47] Dr. Harmathy testified that prior to the accident Ms. Garratt had intermittent back pain, noted once or twice by him, but nothing that he would call chronic. Absent the accident, she would not have ended up in her current condition. He would have expected to see her intermittently for back pain, requiring manipulation, likely caused by working excessively. Dr. Harmathy testified that the back pain with which she presented following the car accident was different in quality than her original back pain. The original pain was related to an almost mechanical dysfunction of the sacroiliac joint, whereas the pain following the accident was more related to stretched and perhaps torn tissues and to muscle dysfunction. Between the accident and the date of trial, Dr. Harmathy had seen Ms. Garratt 36 times, and her back problems were much different.

[48] Dr. Parker opined that Ms. Garratt had some degenerative facet joint disease prior to the accident (based on the CT scan), which placed her at risk of episodic back pain. He provided the opinion that Ms. Garratt did not have chronic back pain prior to the accident - rather she had episodic back pain which could be expected to continue to occur in the future because of her job. In the absence of the accident, however, he would not have anticipated any rapid deterioration in her condition and/or any significant functional limitations or restrictions imposed by this condition until she was well into her sixties. In formulating this opinion, he took into account her work as a cleaner.

[49] Dr. Crohn testified as to Ms. Garratt's pain presentation before the accident. She worked in a physically demanding occupation - lifting, bending, twisting and carrying - the result of which is often wear and tear injuries. Her prior injuries did not, however, affect the quality of her life. She was regarded as a very "optimistic, courageous, very strong person before the accident". After the accident, looking at the whole picture, the character, severity and functional impact of her pain changed dramatically. The pain went from a circumscribed pain to a multi-site chronic pain problem, which significantly altered the quality of her life.

[50] Dr. Hunter testified that he was aware of the prior episodes of back pain and her attendances at the chiropractor. He had reviewed the clinical notes and records of Dr. Munro and Dr. LaFrance. His conclusion from the review was "I don't think these pre-existing problems were significant, and I don't feel they contributed to her current level of apparent dysfunction". I find it puzzling that the defence pursued the theory that her pre-existing injuries had such an impact on Ms. Garratt in view of its own expert's opinion.

[51] The defence makes much of Dr. Munro's noted remark concerning "chronic back pain", suggesting that it describes Ms. Garratt's prior back problems generally. It does not. I accept that she had prior episodic back and neck pain caused by her work. Dr. Parker has provided the only opinion on the timing of the limitations and restrictions that might have been expected to arise as a result of this condition, and I accept his opinion.

OTHER PRE-EXISTING CONDITIONS

[52] Ms. Garratt also suffers from Type II diabetes, which Dr. Harmathy testified is controlled well with a medication called Metformin. Dr. Harmathy explained that the various risk factors and complications that accompany Type II diabetes are less likely to occur when the individual has well-controlled diabetes. The Defendant did not lead any expert evidence on the diabetic risk factors specific to Ms. Garratt, and I accept Dr. Harmathy's opinion that Ms. Garratt's diabetes is reasonably well controlled. There is no evidence on which to find that she will develop these complications.

[53] Ms. Garratt also suffered from Hashimoto's thyroiditis, which developed in mid 2001. She underwent suppression therapy and eventually had her thyroid removed in January of 2004. Dr. Harmathy agreed that thyroiditis could cause fatigue, muscle weakness and depression. He did not offer any opinion that these were manifested in Ms. Garratt nor did the defence call any other expert evidence on this point. Initially there was some trouble controlling her calcium levels post surgery, but eventually they were controlled. Until the calcium levels were resolved (we do not know the exact date), muscles could have gone into spasm in the body. Again, no evidence of linkage to Ms. Garratt's account of muscle spasm was provided.

[54] Defence counsel submits that there should be reductions in non-pecuniary and pecuniary damages, because of these prior medical conditions. The only evidence concerning the diabetes and thyroiditis are general comments about the conditions and the problems that can arise from them. The defence led no opinion evidence establishing that Ms. Garratt is at increased risk, because of the diabetes and thyroiditis, of complications and problems relating to the reduction in functioning for which she is to be compensated. It is not appropriate to make speculative findings in the absence of evidence, therefore no reduction in damages will be made on this basis.

CONCLUSION ON PRE-EXISTING CONDITIONS

[55] In view of all of the medical opinions, I find that on a balance of probabilities Ms. Garratt's pre-existing conditions did not contribute to her post-accident level of dysfunction. The Defendant has not established that her pre-existing problems (neck, back, thyroid, diabetes) would have led to her current problems. Absent the accident, no functional limitations or restrictions would be imposed on Ms. Garratt until well into her sixties. In providing this opinion, Dr. Parker was aware of Ms. Garratt's history of prior neck and back pain as well as her diabetes and thyroiditis.

[56] The crumbling skull doctrine has no application in this case. Its application is in the assessment of damages where an award will be lower in cases of pre-existing frailty and deteriorating conditions that would have manifested themselves in any case, absent the tortious injury (*Athey v. Leonati* and *Blackwater v. Plint* supra). We have the opinion from Dr. Parker that she would not have experienced problems from her back causing limitations and restrictions until well into her sixties. There is no compelling evidence that Ms. Garratt's diabetes or thyroiditis would change that progression of events in any way.

LYNDA GARRATT'S FUTURE PROGNOSIS

[57] Dr. Parker opined that Ms. Garratt, as of July of 2005, reached her maximum medical recovery. He does not anticipate any further improvement.

[58] Dr. Crohn saw Ms. Garratt on June 30, 2005 and opined that, in the absence of an inter-disciplinary approach to treatment, involving her family doctor, other medical specialists, a psychologist, a physiotherapist and others as necessary, her pain will remain unresolved.

[59] The Defendant submits that Ms. Garratt can still expect improvement and therefore has a better future prognosis than that given by the doctor. He submits that Ms. Garratt had success in dealing with the loss of a child at 32 days (to crib death) through the use of hypnotherapy. He submits that because Dr. Crohn has previously used hypnotherapy with success, if Ms. Garratt underwent hypnotherapy, her pain and prognosis would improve. However, Dr. Crohn testified that he does not believe that hypnotherapy would help with her pain at this point. Its use would necessarily be just one aspect of the interdisciplinary treatment discussed above.

[60] I would have thought that in order to satisfy the court that hypnotherapy will realistically provide improvement in Ms. Garratt's chronic pain, this "theory" would have been canvassed with Dr. Hines, the defence expert in psychiatry, and not just put to Dr. Crohn in cross-examination. Counsel did not go so far as to argue that failure to use hypnotherapy represents a failure by Ms. Garratt to mitigate her damages.

[61] On the evidence before me, it appears that Ms. Garratt has reached the highest level of recovery that she can expect to reach, at least in the absence of entering into an inter-disciplinary program at an integrated facility, the closest of which to Ms. Garratt is located likely in Toronto.

MITIGATION

[62] Defence counsel submits that Ms. Garratt's damages for chronic pain, (should I find her to have sustained chronic pain – which I do) should be significantly reduced in light of her failure to enroll in an inter-disciplinary treatment program. I do not agree. First of all, the accident happened in November of 2002. The earliest anyone knew about this potential treatment plan was after Ms. Garratt was seen by Dr. Crohn in June of 2005.

[63] Further, the Defendant has not led any evidence to establish that enrolment in such a program would have had a successful outcome, or in fact any impact on Ms. Garratt. Dr. Crohn testified that he could not comment on whether such a program could deal with her complicated pain symptoms.

[64] In any event, the Plaintiff led evidence establishing that her treatment programs were appropriate.

[65] Dr. Harmathy testified that Ms. Garratt had an open mind about different types of therapy. She tried physiotherapy, reiki, shiatsu massage, and Korean hand therapy. She

consistently saw him, never pushed for medications and she pushed herself to get back into work. She not only pushed herself to comply with his prescribed treatments, but also engaged in pain relieving maneuvers on her own, some of which she paid for herself.

[66] Dr. Hunter testified that in his written report he stated “I am surprised that she is still having symptoms after this time period, and after appropriate rehabilitation, and physiotherapy, and massage therapy, etc.”

[67] The defence relies on the opinion of Dr. Ken Marek, a psychologist, who in September of 2003 felt that Ms. Garratt would benefit from twenty sessions of psychotherapy, aimed at helping her to develop skills to cope with pain as well as the resulting anxiety and depression related to the pain. Ms. Garratt testified that she did not undergo any treatment because she did not believe it would be covered by her insurer. There is no evidence that it would have been covered. The only evidence about treatment came from Linda Mash who testified about the submission of treatment plans.

[68] The defendant submits that Ms. Garratt failed to mitigate her losses because she did not undergo the recommended psychotherapy. I do not make such a finding. Her family doctor did not recommend psychotherapy, and Dr. Hines agreed that it was reasonable for Ms. Garratt to rely on the direction of her doctor regarding psychological treatment.

[69] I also note that Dr. Hines, the defence psychiatrist, who saw Ms. Garratt, believes that she suffers from specific driving phobia and is not suffering from depression. If it is the defence’s position that Ms. Garratt is not suffering from depression, one wonders why it would view psychotherapy as so integral to her treatment.

[70] The onus lies with the Defendant to demonstrate a failure to mitigate. Ms. Garratt, who tried treatment after treatment, followed her doctor’s advice and tried her own forms of treatment, has met her obligations. I find that she has reasonably attempted to mitigate her damages.

PECUNIARY CLAIM FOR MS. GARRATT’S LOST INCOME

[71] Ms. Garratt ran a cleaning business before and after the accident. G.L. & V. is her commercial client. She receives a monthly stipend under their contract and is not responsible for reporting hours. How she accomplished the cleaning did not concern G.L. & V (i.e. by her own labour or employing other workers). She has retained the contract and the loss of income claim, both past and future, is put forward on the basis of the cost of hiring helpers to fulfill that contract.

[72] The medical evidence supports the necessity of Ms. Garratt hiring additional help if she is to retain her contract with G.L. & V.

[73] Ms. Garratt also claims lost income in regards to her inability to continue cleaning in private homes, as she did before the accident.

MR. GARRATT'S HELP

[74] Ms. Garratt's husband helped her at G.L. & V. before and after the accident. There was much time spent on the discrepancies between his recollection as to the hours worked and Ms. Garratt's recollection. I accept Mr. Garratt's recollection, as he was the one volunteering the help. In any event it does not matter, as he was not paid. For a couple of months prior to the accident he would give her a hand if she needed him. The defence attempt to tie Mr. Garratt's help to his wife's neck problem in the summer of 2002 was not successful. Mr. Garratt helped his wife because he was her husband. On many occasions he was just sitting at home and if he could help her, she could get the work done faster. This makes sense.

[75] Mr. Garratt helped his wife following the accident, probably a little bit more, because some of the work was just too heavy for her. Again this makes sense.

[76] Mr. Garratt was not assisting his wife on a formal basis. There will be no deduction taken from the past or future income claim as a result of his assistance which has nothing to do with the necessity of additional workers to help Ms. Garratt fulfill her contract with G.L. & V.

LEGAL PRINCIPLES RELATING TO PAST AND FUTURE LOSSES OF INCOME

[77] The onus to establish a past loss of income lies with the plaintiff. The burden of proof is on the civil standard, which is a balance of probabilities.

[78] It is not necessary for the plaintiff to prove on a balance of probabilities that a future pecuniary loss will occur. In *Graham v. Rourke* (1990) 75 O.R.(2d) 622 (C.A.) Doherty J.A. wrote:

A trial judge who is called upon to assess future pecuniary loss is of necessity engaged in a somewhat speculative exercise... A plaintiff who seeks compensation for future pecuniary loss need not prove on a balance of probabilities that (his) future capacity will be lost or diminished... if the plaintiff establishes a real and substantial risk of future pecuniary loss, (he) is entitled to compensation...

[79] In *Schrump v. Koot* the Court of Appeal of Ontario stated as follows:

In this area of the law relating to the assessment of damages for physical injury, one must appreciate that though it may be necessary for a plaintiff to prove, on a balance of probabilities, that the tortious act or omission was the effective cause of the harm suffered, it is not necessary for him to prove, on the balance of probabilities, that future loss or damage will occur, but only that there is a reasonable chance of such loss or damage occurring. (*Schrump v. Koot* (1977) 18 O.R. (2d) 337.

[80] This principle was confirmed in *Giannone v. Weinberg* where it was stated:

It is not necessary for the plaintiff to prove on a balance of probabilities that a future pecuniary loss will occur but only that there is a reasonable chance of such loss occurring. The court must then assess the value of that chance...it must be remembered, however, that in determining the future pecuniary loss, which at best must be an estimate, the court should take into consideration the evidence of both positive and negative contingencies which may affect that loss. (*Giannone v. Weinberg* (1989) 68 O.R. (2d) 767 (C. A.).

[81] The injured party is to be restored to the position he/she would be in had the accident not occurred. Where the evidence permits, the court should compare what the plaintiff would have earned had he/she not been injured with what he/she will earn in his/her injured state. In addition, in recognition of the fact that the future cannot be foretold, an allowance can be made for the contingency that the assumptions on which the award for pecuniary loss is predicated may prove inaccurate. Where no evidence is available, the courts have often made a deduction for such matters in the range of 20%. Where evidence is available, the deduction for contingencies may be increased, decreased or eliminated, or an increase for contingencies may be warranted, according to the facts presented.

[82] Evidence was adduced regarding the specific issue of Ms. Garratt's lower extremity problems, and a deduction will be made on that basis. There will also be a deduction in the amount of 20% to address the reasonable chance of general contingencies such as sickness, accident, death, holidays, and loss of contracts.

[83] As the Supreme Court noted in *Engel v. Salyn* [1993] 1 S.C.R. 306,

In the case of self-employed persons, quantifying the victim's contribution to the business is inherently difficult. The best approach to calculating future losses must be dictated by the particular circumstances. Expert witnesses may assist the judge in determining the most appropriate method of calculation.

In the case of wage earners, forecasts are more easily based on the salary commanded before the date of the accident. However, where the injured party is self-employed, the decrease in salary may not be the appropriate measure of damages, where the paid salary does not reflect the victim's actual contribution to the business. Similarly, a loss of profits approach may be warranted in some cases, and not in others. In the case at bar, no evidence of loss of profits was presented, other than the loss attributable to the cost of replacement labour.

[84] The cost of replacement labour is also the proposed method of valuing Ms. Garratt's income losses in respect of her contract with G.L. & V. In this case, that is an appropriate measure of those damages.

COST OF THE PAST EXTRA HELP

[85] Prior to the accident Ms. Garratt worked about 35 hours per week for G.L. & V. Her mother, Patricia Mitchell, worked two hours per day, five days a week at \$10 per hour. Ms. Garratt paid the income taxes on that wage. The plaintiffs' expert, Michael Kavanagh, an actuarial valuator, provided his calculation to establish the total amount per hour, factoring in the taxes paid by Ms. Garratt - \$12.83 per hour (calculated to an annual rate of \$3,336.00). That calculation was not disputed by the defence. All of Mr. Kavanagh's calculations were made up to October 2005, the start of this trial.

[86] Following the accident, Mrs. Mitchell worked an additional two hours a day. Mrs. Mitchell was only able to work until the end of December 2002 because of health reasons. She was replaced by Rhonda Gendron, who worked at the same rate and hours as Mrs. Mitchell. Ms. Gendron worked until the end of February 2003, at which time she was replaced by Ms. Garratt's sister, Donna Johnston. Ms. Johnston's hours varied but generally she worked three hours a day, five days a week. Ms. Johnston was initially paid \$10 an hour with Ms. Garratt paying her taxes, however, starting April 1, 2005, she received \$11.25 an hour but was paying her own taxes.

[87] Mr. Kavanagh's report summarizes this additional help as follows:

Employee Extra Payment Cost Past Loss

Beginning Date	Ending Date	Extra Daily Hours Paid	Hourly Pay Rate	Annual Rate	Individual
02-Nov-02	31-Dec-02	2 hours	\$12.83	\$6,672	Patricia
01-Jan-03	28-Feb-03	2 hours	\$12.83	\$6,672	Rhonda
01-Mar-03	30-Mar-05	1 hour	\$12.83	\$3,336	Donna
01-Apr-05	11-Oct-05	1 hour	\$11.25	\$2,925	Donna

[88] Mr. Kavanagh used this data to compute the past loss to October 11, 2005.

[89] After factoring in various contributions Ms. Garratt was required to make as an employer, the past loss is \$6,549.00. I accept this amount as having been proven on a balance of probabilities.

COST OF THE FUTURE EXTRA HELP

[90] The future loss relating to the contract with G.L. & V. is comprised of two parts, the loss associated with the additional work for the extra workers and the loss beginning at the time Mr. Lima is no longer working without compensation. Mr. Kavanagh's calculations were made on the basis that Ms. Garratt will retire at age sixty-five. She testified and I accept that this was her planned retirement date.

[91] Ms. Garratt testified as to the problems with her lower extremities that arose in 2004 and affect her work. I have found that these problems were not caused by the accident. She has a physically demanding job that requires and will continue to require significant use of her knees. There shall be a specific contingency deduction of 15% to factor in the impact of her lower extremity problems.

EXTRA EMPLOYEE

[92] The capitalized amount for the future cost of the extra employee calculated by Mr. Kavanagh from October 11, 2005 is \$41,866. Mr. Kavanagh's figure will be reduced by 20% to reflect the general contingencies discussed above and by a further 15% to reflect Ms. Garratt's lower extremity problems to \$27,212.90.

MANUEL LIMA

[93] Manuel Lima has been helping Ms. Garratt 3½ to 4 hours per day, five days per week on a non-paid basis. He is a long-standing friend of Ms. Garratt who, as of the end of January 2006, will be unable to continue with the help because of his age and health. He is seventy-one years of age. On October 27, 2005 he testified that he would only be continuing with his help for two or three more months because he has been doing the heavy work for her and it is hard for him to get in and out of the car at the end of the shift to go home. Ms. Garratt will have to pay someone else to fill in for Mr. Lima. The rate of this loss as calculated by Mr. Kavanagh is \$9,750 per year.

[94] The capitalized amount for that claim running from February 1, 2006 calculated by Mr. Kavanagh is \$135,320.00 (February 1, 2006 to age 65). Mr. Kavanagh's figure will be reduced by 20% to reflect the general contingencies addressed above and by a further 15% to reflect Ms. Garratt's lower extremity problems - to \$87,958.

PRIVATE HOUSEKEEPING CLIENTS

[95] Ms. Garratt also had private housekeeping clients prior to the accident who include the following:

- (a) Karen Wilford for whom she cleaned every week at \$50 per attendance;
- (b) Bernice Vernon for whom she cleaned every other week at \$75 per attendance;
- (c) Mrs. Bullen for whom she cleaned every other week at \$48 per attendance;
- (d) The Perryman residence at which she cleaned every other week at \$30 per attendance.

[96] She has been able to continue cleaning at the Perryman residence but has given up the other three jobs.

[97] The medical evidence supports Ms. Garratt's inability of returning to this private client work. Dr. Parker opined that Ms. Garratt has reached her maximal medical recovery. Dr. Crohn says her work ability is like a bank – she can only draw on so much energy and she is using it for G.L. & V.

[98] Mr. Kavanagh calculated the annual amount of loss to be \$5,798 per year, arriving at a capitalized amount of \$82,987. Ms. Garratt's estimate was \$4,600 per year, which includes the Perryman residence. I accept Ms. Garratt's evidence as to the \$4,600 per year and I am also deducting \$780 per year, representing the Perryman work, and have calculated her annual loss from private clients at the amount of \$3,820 per year. This will be reduced by 20% to reflect the general contingencies discussed above and by a further 15% to reflect Ms. Garratt's lower extremity problems – to a capitalized amount of \$35,539.18

FUTURE COST OF HOUSEKEEPING AND HOME MAINTENANCE

[99] Stephen Glenney, an occupational therapist, testified and provided information about the future cost of housekeeping and home maintenance to Ms. Garratt. He provided evidence as to Ms. Garratt's functioning and made recommendations about required housekeeping and home maintenance services. Mr. Kavanagh incorporated this evidence into his calculations.

[100] The evidence has established that Ms. Garratt's pre-existing back and neck difficulties did not present a significant functional barrier to her working. In addition, before the accident, she maintained her own house. Following the accident she stopped cleaning the private homes and had to hire help at G.L. & V. Her husband has taken over some of her housekeeping chores. Mr. Glenney's conclusion is that she has a reduced ability to independently and consistently complete her housework and do her various work tasks. I accept his opinion.

[101] Mr. Glenney performed testing on Ms. Garratt. Manual muscle testing and active range of motion tests yielded results mostly within functional limits.

[102] Mr. Glenney also took us through his findings relating to her normal daily activities. On many tasks, she has gone from independence to modified or partial independence following the accident. This is the case in regards to mobility, grocery shopping, meal prep and clean up and housekeeping. On many of the more strenuous activities, her level of ability has degraded to dependent functioning.

[103] Loss of future homemaking capacity is a separate head of pecuniary damage, the purpose of which is to compensate for the loss of the value of the work that would have been performed by the plaintiff. The Ontario Court of Appeal has characterized the loss of capacity to work as a negative loss (*Payne v. Alb* [1999] O.J. No. 1954). Different approaches have been taken to the valuation of this loss. The fact that domestic labour and management has value has been firmly established in Canadian law (*Peter v. Beblow* [1993] 1 S.C.R. 980), however quantification of that value is often difficult. In this case, there is evidence regarding the cost of replacing Ms. Garratt's labour in the home, providing the most satisfactory means of valuing her lost capacity in this case.

[104] Mr. Glenney's report was not made an exhibit, however Appendix "A" to his report was made an exhibit on consent and sets out data regarding the equipment and services, which was then considered by Mr. Kavanagh. This Appendix is attached to this judgment as Schedule A.

[105] Mr. Kavanaugh assumed the mid-point where Mr. Glenney has expressed a range of values. Because frequency was not provided for lawn care or snow removal, he estimated those needs as occurring ten times a year. This was not disputed by defence counsel and makes sense. Plaintiff's counsel suggested that the frequency should be increased, but I decline to do that.

[106] Mr. Kavanaugh has calculated the capitalized amount of these expenses to be \$31,868.00. This will be reduced by 20% to reflect the general contingencies discussed above and a further 15% to reflect the lower extremity problems – to \$20,714.20.

SUMMARY OF PECUNIARY DAMAGES

[107] To summarize, the following pecuniary claims have been established:

- (e) past extra employee cost of \$6,549.00;
- (f) future extra employee cost of \$27,212.90;
- (g) future replacement cost of Mr. Lima of \$87,958;
- (h) future loss of private clients of \$35,539.18;
- (i) future housekeeping and house maintenance expenses of \$20,714.20;

TOTAL: \$177,973.28.

GENERAL DAMAGES

[108] Ms. Garratt has problems with her neck and back. She has headaches and is depressed. She does not have the same amount of energy. She has developed chronic pain, which has affected her life dramatically and made her a totally different person. The witnesses – doctors, family, friends, and clients have been consistent – she is a totally different person since the accident.

[109] Ms. Garratt's chronic pain and disability have seriously inconvenienced her. She has had numerous attendances with multiple healthcare providers. Her back pain has reached maximal medical recovery and her pain is constant and disabling. Her problems are particularly disabling because her job requires much lifting and standing. The consequences of these injuries have had, have and will continue to have an effect on Ms. Garratt's health and ability to enjoy the normal activities and pleasures of life, including recreational, social and work-related activities. Both Ms. Garratt and the other lay witnesses talked extensively of the significance of dancing to Ms. Garratt and the effect of the loss of that activity on her life. Her general damages are assessed in the amount of \$75,000.

[110] Ms. Garratt confirmed that the problems with her lower extremities affect her work and social activities. She testified as to the importance of those activities to her, particularly the dancing. Her general damages shall be reduced by 15% to reflect the impact of her lower extremity problems on these activities – to \$63,750.

FAMILY LAW DAMAGES

[111] Bill Garratt is Lynda Garratt's husband whom she married on August 25, 2001. He testified in a forthright manner. Prior to the accident they had an active life – socially they went dancing at least two nights a week. They enjoyed walking and camping. His wife hosted all of the extended family events. Since the accident, his wife has been emotional and forgetful. They enjoyed an active sex life prior to the accident which, since the accident is non-existent. The accident has caused quite a bit of stress in their relationship. He almost left Lynda a couple of times but hopes that their marriage will survive. He has suffered a loss of care guidance and companionship. I assess his claim at \$20,000. His wife's problems with her lower extremities will have an effect on him and there shall be a contingency deduction of 15% to reflect their impact – to \$17,000.

INSURANCE ACT

[112] There is no statutory reduction in damages, in part because this Defendant is not afforded protection under the *Insurance Act* R.S.O. 1990, Chapter I.8. There will also be no deduction with respect to the income or any other benefits received by Ms. Garratt as her injuries did not constitute "loss or damage from bodily injury or death arising directly or indirectly from the use or operation of an automobile" as required by the *Insurance Act*.

[113] Collateral benefit deductions pursuant to the *Insurance Act* are only available in cases where the damages arise “directly or indirectly from the use or operation of an automobile”. The case law interpreting this and other like provisions distinguishes instances where the plaintiff’s injuries arose out of the use of a vehicle from those where the vehicle is incidental to the defendant’s negligence. The Ontario Court of Appeal set out the test to be used in applying the statutory requirement to particular facts. The question is whether

the plaintiff’s “loss or damage”, as opposed to his injuries, were caused by the use of his car, or whether the “connection between the [loss or damage] and the ownership, use or operation of the vehicle [was] merely incidental or fortuitous” (Hernandez v. 1206625 Ont. Inc. (2002), 61 O.R. (3d) 584).

[114] Here, the use of the car did not cause Ms. Garratt’s losses. As determined by the causation analysis above, those damages were caused by the defendant’s negligence. Had Ms. Garratt been walking instead of driving under the overpass, she might still have been injured.

SUMMARY OF DAMAGES

[115] The following are awarded:

- (i) Pecuniary - \$177,973.28
- (ii) General - \$ 63,750.00
- (iii) FLA – husband - \$ 17,000.00

TOTAL = \$258,723.28

[116] Prejudgment interest is awarded. If the parties cannot agree on costs, the trial co-coordinator can be contacted to arrange for submissions, which can be made in writing or orally. If any mathematical error has been discovered in these Reasons for Judgment, it can be brought to my attention.

J.E. Ferguson J.

Released: April 13, 2006

